

Stroke (Cerebrovascular Accident)

History

- What are symptoms? – One-sided weakness, facial droop, speech slurring or aphasia (name objects), altered mental status, headache
- When did symptoms start? – Try to get as much detail as possible
 - Does patient / family have an exact time of onset?
 - Did patient wake up with symptoms? This makes the time of onset less certain.
 - What was patient doing when symptoms started?
- Does patient have a history of seizures, diabetes, hypertension, myocardial infarction, recent trauma or surgery, bleeding disorder, previous stroke or transient ischemic attack (TIA) “mini-stroke”
- Is the patient on medications? Especially insulin, aspirin, Plavix, anticoagulants, or antihypertensives?
- What is the patient’s baseline mental status and activity? – Is the patient bedridden or wheelchair bound? Does the patient have dementia?

Physical Exam

- ABC’s / Vital Signs – Is the patient maintaining airway? Is the patient hyper- or hypotensive? Is the patient hypoxic?
- Mental status exam – Is the patient obtunded or unresponsive?
- Blood glucose – Remember hypoglycemia is a great mimicker of stroke!

Stroke Exam

• BEFAST

- Balance- dizzy, headache
- Eyes-visual disturbance
- Face-drooping
- Arms-or leg weakness
- Speech-difficulty
- Time-Last known well

VAN Large Vessel Occlusion (LVO) Stroke Exam

- Unilateral Arm weakness (required) and at least one of the following
 - Vision deficit /fixed gaze
 - Aphasia (not slurred speech)
 - Neglect

Primary Stroke Center–(PSC) BEFAST Stroke Alert Criteria

- Last known well within 6 hours
- Any abnormal finding on BEFAST Exam
- Blood glucose within 60-600

OR

- VAN + and LKW less than 6 hrs
- VAN + with Last known well between 6 hours and 24 hours
- VAN + Stroke upon awakening
 - Patient taking anticoagulants and VAN PLUS
 - Stroke patient with GCS < 13, needing intubation or blown pupil

EMT/AEMT

- Follow General Medical Care Guideline
- Follow Adult Resuscitation Guideline, as indicated
- Perform **BEFAST** Exam, Record Last Known Well (LKW) time, Witness Name and Cell Number
- Prepare for Transport Nothing by Mouth (NPO)

Paramedic

- Advanced airway as indicated
- Stroke Alert if meets Criteria
- Limit scene time to life-saving interventions
 - Notify Medical Leader of patient meeting stroke alert criteria to assist with evacuation plans
- Keep Head of bed elevated to 30° if SBP >110
- **NS/LR** 500 – 1000 mL IV/IO to maintain SBP >110
- 12- Lead EKG
- Give witness name and cell number to transporting ambulance staff